Artificial Intelligence and User-Centred Design at NHS England

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Artificial Intelligence and User-Centred Design at NHS England

Launched in February 2026, this hub includes the User-Centred Design (UCD) policy and guidance for Artificial Intelligence (AI) usage at NHS England.

Artificial intelligence policy and guidance

- Key definitions
- Principles when using AI
- Risks of AI use
- Benefits of AI

Artificial intelligence tools

- Approved tools
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If your team are **building a new service with AI, or integrating AI,** read:

Standards to follow

- UK government guidance
- NHS guidance

Transparency and Assurance

- Assurance processes
- Information governance
- Ask for a technical review or second opinion

Learning and development resources

- Training events
- Reading, videos and courses

Get involved sharing good practice

- Slack channels
- Community meets-ups
- AI for UR Working Group
- UCD AI Prompt Library
- Case studies
- Ongoing Experiments
- UCD AI backlog

Help us improve this guidance

Share insights or feedback and take part in the discussion. We use GitHub as a collaboration space. All the information on it is open to the public.

Feed back or share insights on GitHub

Read more about how to feed back or share insights.

If you have any questions, get in touch with the service manual team.

AI hub

Artificial intelligence policy and guidance

Introduction

Artificial Intelligence (AI) is reshaping research design, offering new ways to analyse data, prototype, test ideas, learn and streamline workflows.

The UCD AI hub combines User-centred design policy and guidance for Artificial Intelligence usage at NHS England.

This policy applies to ALL user-centred design staff (permanent, contractor, agency or Scheme of Work) who work on any NHS services.

This resource is continually evolving. Some topics have limited information, but this will expand as more teams use AI tools and share what they learn.

This guidance explains how to responsibly use AI, supporting teams to:

- Gain and maintain AI literacy.
- Foster creativity and innovation.
- Improve efficiency without sacrificing principles.
- Ensure ethical, safe, and inclusive outcomes.
- Adhere to user-centred design standards.

Scope of this guidance does not cover:

- How to choose, design, or implement AI tools as part of a service to serve the end user.
- Production code development (see AI (dev tools) User Guide).
- Agentic AI use for user research.
- How to use AI as a tool for wider design practice, e.g. service design, or creation of design artefacts.

Key definitions

To help you better understand this guidance, please review these key definitions, which are taken from the NHS AI Dictionary and the AI Playbook for the UK Government.

Artificial Intelligence: the use of digital technology to create systems capable of performing tasks commonly thought to require human intelligence.

Generative artificial intelligence:

- A subset of AI capable of generating text, images, video or other forms of output by using probabilistic models trained across various domains.
- Generative AI learns from large amounts of specially curated training data to discern and replicate complex patterns and structures.
- The output of generative AI models mimics the characteristics learned from the training data, enabling a range of novel applications; these include personalised content generation, advanced analysis and evaluation, and aiding creative processes.

Agentic artificial intelligence: autonomous AI systems that can make decisions and perform actions with minimal human intervention. These agents are capable of understanding their environment, identifying the set of tools and functions at their disposal, and using those to take actions to achieve their objectives.

Large language model (LLM): a neural network that is trained on a vast amount of text. The training uses unlabelled text and some form of self-supervised learning.

Principles when using AI

Whenever you use AI, it is essential to:

- **Comply** with policy and guidance, including NHS England user-centred design best practices, principles, and standards. Using AI in your design practice must comply with and not contradict the following standards and principles.
- **Learn** continuously. Develop skills in using AI, for example by learning prompt strategies and understanding its limitations.
- **Document** how and where AI was used in the research and design process transparently.
- **Enhance**, not replace, human processes; you remain accountable for all outputs generated by AI. AI cannot take responsibility for decisions or outcomes.
- Apply **human** judgement, empathy, and contextual awareness. Do not rely on AI for key human tasks, such as interviewing users or making recommendations.
- **Verify**. Always fact-check and sense-check outputs before use, for example AI-generated outputs for inclusion, bias, and suitability.
- **Measure** costs, including environmental impact. AI tools generally consume more energy than traditional methods. (See: unep.org/news-and-stories/story/ai-has-environmental-problem-heres-what-world-can-do-about).
- **Evidence** is a key part of the user-centred design process. Use real primary user data and verifiable secondary sources. Avoid AI-generated data from unknown sources.

- **Share.** Be transparent about AI usage and always document all AI usage in the design process to ensure transparency. Share best practice openly and generously with peers to build collective knowledge.

Risks of AI use

Using AI for user research or design raises several possible risks and issues:

- **Lack of explainability:** AI outputs can be difficult to interpret, reducing the credibility of our work. For example, code generated by an AI tool may work, but contain vulnerabilities.
- **Lack of transparency:** we often cannot verify what data the AI was trained on, how its algorithm works, how it reached its conclusions, and if this happened based on the data we fed it in our query.
- **Risk of inaccurate information:** AI may provide false or misleading outputs (known as "hallucinations"), and it can be hard to assess their reliability. Generative AI models predict and generate content by determining the most likely subsequent pattern based on previous training. They are very good at generating plausible predictions that look correct but may not actually be correct.
- **Bias and inappropriate content:** AI is known to reflect or amplify biases present in training data, leading to outputs that may be discriminatory, offensive, or not inclusive. AI may not always follow accessibility and inclusion best practices, even if trained, or prompted, to do so.
- **Potential disconnect from reality:** If analysis is done primarily by AI, you may not develop deep familiarity with the data. This makes it harder to interpret findings accurately, articulate meaningful recommendations, and present them convincingly to stakeholders. As UCD professionals, full understanding of the underlying data is an essential part of our work.
- **High financial cost:** Procuring, building, and maintaining AI tools can be expensive.
- **Environmental impact:** Generative AI consumes significant energy and resources. When making the decision to use AI, we should consider whether the envisioned benefits of AI justify this impact. Always use AI purposefully rather than casually.
- **Confidential or personal data exposure:** Entering sensitive data into AI tools can result in data breaches, violating NHS Data Protection Policy and legislation.
- **Use of non-approved tools:** Non-approved tools may process, store, or use data in ways that are not secure or compliant. They may lack necessary guardrails, increasing risk of data loss, privacy breaches, or legal issues. Do not input sensitive data into or use outputs from unknown sources.
- **Legal/compliance risks:** Generative AI models may be trained on unlicensed content generating copyright risks for NHSE.

- **Skills gap and deterioration when reliant on AI tools:** Without proper training, staff may misuse AI tools, increasing risk of poor outcomes, laziness or data breaches. Also, UCD skills may be lost through lack of practice.

Benefits of AI use

Using AI for research and design can offer several advantages when applied responsibly:

- **Saving time:** AI can automate repetitive tasks, freeing colleagues to focus on high-value activities (i.e., sharing findings, influencing decisions, and advocating for recommendations with stakeholders) so that our work has maximum impact.
- **Reducing human error:** AI can help minimise manual mistakes by checking consistency in coding and flagging anomalies.
- **Improving clarity and consistency in write-ups:** AI can assist with proofreading, correcting grammatical errors, and ensuring consistent tone and terminology across reports and presentations.
- **Scaling analysis:** AI can process large volumes of qualitative or quantitative data quickly, making it easier to have a draft overview of potential outputs in complex projects or tight timelines.
- **Generating new ideas:** AI can support brainstorming and critical thinking during analysis by suggesting alternative perspectives, highlighting patterns, and surfacing connections that might not be immediately obvious.

AI hub

Artificial intelligence tools

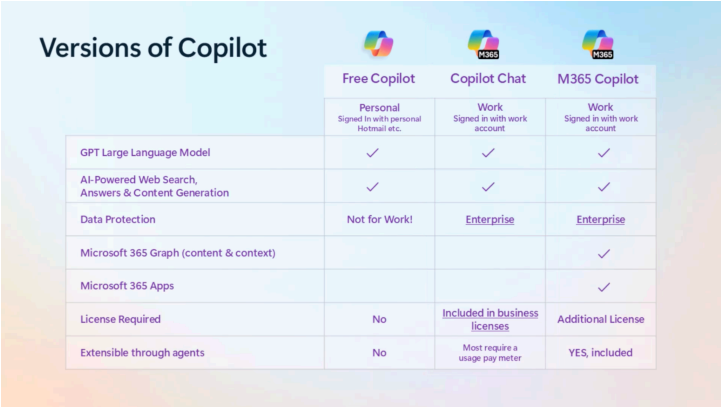
Key points:

- **Only use NHS England approved tools.**
- This policy applies to ALL user-centred design staff (permanent, contractor, agency or Scheme of Work) who work on any NHS services.
- **Never enter personal, personal identifiable data, or business critical information into any AI tool.** This ensures compliance with IT use policies, engineering standards, accessibility requirements, and enterprise guidelines.
- Access to generative AI websites is restricted on corporate NHSE machines (view acceptable use policy), with the exception of Copilot.
  You *may* override the "AI - Site Access Request" warning, however you accept the principles and terms of usage.

Approved tools

General purpose LLM/AI tools:

Tool	Description
Microsoft Copilot	Copilot is Microsoft's AI-powered virtual assistant. It uses large language models (LLMs) to answer your questions using a prompt and response interaction. There are different types of Copilots available at NHSE. For more information on how to use these tools, please see the **Copilot training** running at the moment which goes through this. **Microsoft 365 Copilot Chat**: an AI prompt and response experience that's grounded in the web. • It is available on the web and in Microsoft Edge and integrated in apps like Teams and Outlook. • It doesn't deeply integrate with Microsoft 365 apps for advanced tasks. • Everyone working in NHSE can access it.

Tool	Description
Microsoft 365 Copilot	**Microsoft 365 Copilot desktop application:** includes Copilot Chat functionality plus additional features. • It includes deep integration with Word, Excel, PowerPoint, Outlook and Teams. Access to your organisation's emails, calendars, OneDrive and SharePoint. • Tech Services are working to roll out Copilot 365 to the whole of the organisation. There are a limited number of free licenses available until 31 March 2027. These are being prioritised for permanent NHSE staff. • If you do not yet have an M365 Copilot license, contact: england.copilotrollout@nhs.net to request one, they may need your teams budget code for future recharges.  Comparison table for the different versions of Copilot
Microsoft Copilot Studio	**Microsoft's** low-code graphical tool that you can use to create agents and connect to other data sources. • Agents let you customise your organisation's Copilot experience. You can add existing agents already created by others and create your own agents. • *Currently being trialled only by the Central UCD team - if you want to know more about this please contact them at ucd@nhs.net.*

Tool	Description
Microsoft PowerAutomate	**Azure AI Builder:** Advanced automation tool that supports workflows with AI features • Allows workflows with AI to be deployed alongside OneDrive, SharePoint and Dataverse • Evaluate content in documents (e.g. Word, Excel, PowerPoint, PDFs) • Integrates with Microsoft tools including: PowerApps, PowerBI, PowerAutomate Desktop, Copilot Studio • Requires a premium PowerApps licenses and PowerApps environment which is requested via Tech Services • Guidance available here: PowerPlatformHub & Use AI Builder in Power Automate

Tools and features supporting specific workflows:

Writing and transcription	
Microsoft Word	Microsoft Word has functionality that can help improve content without the need for AI. Microsoft Editor can assist you with spelling and grammar checking, and polishing your writing style. Also has a transcription service - you upload the file, and it generates a high-quality transcript. This is regarded as less accurate than the MS Teams transcription functionality.
Microsoft Teams	Produces transcripts when calls are recorded and the transcription is turned on. It is OK to continue to use this but please don't upload these full transcripts into AI tools, as stated previously in this guidance.
Data analysis and summarisation	
Microsoft Teams - AI Recap	Intelligent call recap on Microsoft Teams creates AI-generated recaps of recorded calls. After the meeting has ended you can access AI meeting notes and recommended tasks through the 'Recap' tab. Our current stance is that NHSE staff can generate a recap of user research sessions. However, researchers should not use these recaps to form part of their analysis. Researchers are advised to be cautious when using recaps for research because the tool may miss nuance, non-verbal interactions and context.
Qualtrics	Qualtrics is the only NHSE approved survey tool. Qualtrics offers a variety of AI features, however many of these are not currently available in NHSE. **Qualtrics TextiQ** is the Qualtics text analysis tool. It allows you to assign topics to feedback received, perform sentiment analysis, and report results with dynamic widgets. If you have a Qualtrics license, you can use TextiQ for analysing open ended questions. On surveys with over 500 submissions, TextiQ can suggest categories and automatically tag data as the survey gets more responses.

Writing and transcription	
Optimal Workshop AI	Optimal Workshop has AI features to assist with simplifying questions and analysis. Refer to the principles around using AI for User Research when performing any kind of analysis. The **"question simplification" feature** can help when you are struggling with phrasing or how to phrase a sensitive topic and colleagues are not available. However, please ensure you do not use this as a default mechanism to check your questions. Researchers will be much better at recommending nuanced, unambiguous language to use.
Prototyping	
Figma Make	Figma Make is an AI-feature that can create a prototypes using Prompts rather than manually creating prototypes.
GitHub Copilot	GitHub's automated code-review and in-line suggestions tool. GitHub is listed in the approved tools for hosting in the Tech Radar.

Safe exploration and evaluation of new AI tools

To support learning, development and innovation, we do encourage safe exploration and evaluation of new AI tools. However, this must be done in accordance with our IT policies, within a controlled process with clear evaluation criteria, and e.g. only use data already available on the public internet, not corporate or NHS data (this includes your own notes and documents).

For example:

- Researchers compared the performance of ChatGPT vs. other LLMs for extracting data from interviews. The data used for the purpose of this evaluation were case studies already available on the public internet.
- A designer needed some artwork to include in an external presentation. They used general prompts that did not contain any corporate data to ask the AI to generate suitable background images. They also checked with the AI that it did not break any Copyright issues before using it.

How to choose an AI tool?

AI models are trained using data. AI models can suffer from data leakage, depending on their intended mode of operation.

The Artificial Intelligence Playbook for the UK government recommends asking the following key questions when deciding which AI tool to use:

- Where is your data being sent and how is it being processed?
- Is your data being used to train future models?
- How long is your data being retained?

Using a new tool

Before implementing or trialling any product, or if you want to procure enterprise licenses for an AI tool, you need to follow the new software request process. Getting new software approved, especially if it is paid, will likely involve quite a lengthy process which includes getting approval from:

- Getting Approval To Spend (GATS)
- Business Case (Financial and Commercial)
- Information Governance
- Cyber-security

Tools that are not Approved

- **All tools used for NHS work** must be approved by Information Governance (IG), Cybersecurity, and Finance, to ensure NHSE compliance and security.
- This is not an exhaustive list. Tools that are not listed should not be considered automatically approved. We have focused on the most common tools that are relevant to research and design.

ChatGPT (OpenAI)

ChatGPT (free)

You cannot use the free version of ChatGPT when doing any type of NHSE-related work. The tool is not supported by NHS IT. There is currently a lack of transparency over how data is being used or stored. Unless you opt out, inputted data will be used to train future versions of the model.

ChatGPT Enterprise (paid)

The Enterprise version is more secure compared to the free version. It includes Enterprise-level security and controls, System for Cross-domain Identity Management, user analytics, domain verification, and role-based access controls.

However, there are currently no Enterprise licenses available in NHSE for ChatGPT. If you want to procure Enterprise licenses you need to follow the new software request process.

NotebookLM (Google)

Grounds its responses only in the documents, web pages, or videos you upload. It highlights exact references in your materials, showing where information came from. It should therefore avoid hallucinations by sticking strictly to your content.

NotebookLM (free)

You cannot use the free version of NotebookLM when doing any type of NHSE-related work. The tool is not supported by NHS IT. NotebookLM does not use personal data to train the Gemini model.

NotebookLM for enterprise (paid) - Via procurement process

The Enterprise version is more secure compared to the free version. It includes additional privacy and security features like Virtual Private Cloud Service Controls to protect sensitive data from unauthorized access or data exfiltration. It also includes Identity and Access Management controls which can make sure that the right people have the right access at the right time.

However, there are currently no Enterprise licenses available in NHSE for NotebookLM.

Microsoft Clipchamp

Clipchamp is currently not permitted for use because it includes AI-powered features, and using it would involve sharing personal data with an AI service. If you need to edit a video, you can use DaVinci Resolve, which is available through NHSE's Company Portal.

Mural AI

Mural is an approved whiteboard tool in NHSE, together with Microsoft Whiteboard. Mural has some incorporated AI features. However, these AI features are not currently available in NHSE.

GitHub CoPilot

Personal paid accounts are not approved due to data security risks and insufficient enterprise safeguards. See process for requesting an enterprise GitHub Copilot account.

Claude.ai

Personal paid accounts are not approved due to data security risks and insufficient enterprise safeguards.

Get started with GitHub Copilot

You may use GitHub Copilot to assist you in prototyping in code or for processing anonymised research data. It is an automated code-review and in-line suggestions tool. GitHub is listed in the approved tools for hosting code in the Tech Radar.

This is optional, and can be used alongside other AI/LLM tools and methodologies.

GitHub Copilot is the approved platform for AI-assisted coding. Other standalone coding assistants, such as Claude Code and Codex, are not approved for use. Follow the safeguards described by the Engineering Centre of Expertise in the guidance linked below.

Coding… this is all new to me

Coding environments are frequently used in design roles, academic research, quantitative research and data science and analytics teams. However, for many user researchers and colleagues across the wider organisation, they may never have needed to use a coding environment / integrated development environment (IDE) before.

It can therefore be quite a steep learning curve, however there are lots of good resources available to support individuals:

1. It is useful to know some coding basics (HTML, CSS, JavaScript, Python and PowerShell) so you can understand Copilot's actions and output.

   Review coding and AI courses on Pluralsight, the Workforce Training and Education site or YouTube.

2. If you haven't used GitHub before, watch these training videos.

   Also familiarise yourself with Guidelines When Using GitHub.

   Further information on GitHub, including how to get access and set it up correctly is available at: Store code online with GitHub - NHS prototype kit or at Source Control.

3. It is also useful to learn effective prompting to get AI to generate the results you need.

   Tip: You can use Microsoft Copilot and GitHub Copilot to support learning how to code and write good prompts (meta prompting).

Step-by-step approach to setting up GitHub Copilot environment

1. Start by creating a GitHub account with your @nhs.net email address.

2. Then request GitHub Organisational access (e.g. to NHS Digital, NHS.uk, NHS England Tools).
3. Then request a GitHub Copilot licence. See GitHub Users and the AI Coding Assistants - User Guide. Contact the GitHub Admin team mailbox with the required information.
4. Next, install the required software. If you are on a Corporate Managed device, these are available in the Company Portal:
   - X24 - Visual Studio Code
   - X24 - GIT for Windows
   - X24 - GitHub Desktop (version 3.4.+)
5. NHS corporate managed device users need to create a "DevWspace" folder:

   ```
   mkdir "C:\Users\SHORTCODE\OneDrive - NHS\DevWspace\"
   ```

   Your projects and repositories must be stored inside the DevWspace folder. This is the only folder that allows scripts to run.
6. Then review the further steps on the Copilot Starter Pack guidance document: Copilot Starter Pack-v14-20260910_110842

Choosing which model to use

GitHub Copilot supports a range of models (Anthropic/Microsoft/OpenAI), each with their own capabilities and risks.

Check GitHub guidance for Supported AI models in GitHub Copilot - GitHub Docs.

If you are unsure where to start, use the "/model auto" setting, it is cheaper (discounted) and will usually select a suitable model for your task.

Claude models are often considered better for coding. However there is some personal preference, and models do change regularly.

Additional software dependencies

Depending on the tasks you plan to complete, you may need to install further software that integrates with VS Code.

1. If you plan to use GitHub Copilot with the NHS Prototype Kit, install:
   - X24 - NodeJS LTS (version 22+)
2. If you plan to use GitHub Copilot for research data processing, install:
   - X24 - Python (version 3.12+)
   - X24 - Visual Studio Code: Python Extension
   - X24 - Quarto (version 1.4+) - for publishing Markdown documents as HTML pages or GitHub sites

Sandbox your Copilot

This is advanced-user guidance only. Follow this if you need to run GitHub Copilot in a secure sandbox environment.

A sandbox prevents Copilot from accessing any files outside an isolated container. It helps prevent your system settings and files from accidentally being changed by an autonomous agent.

It can also be useful when testing locally installed frontier AI or language models, including in offline mode. However, the GitHub Copilot guidance says to use only approved LLMs. Do not run open-source, local or otherwise unapproved LLMs through tools such as Ollama or LM Studio.

Before starting, be aware that installing WSL, Docker and dev containers use a large amount of disk storage. Required packages can increase this by 20 GB or more. This setup is not recommended on an NHS corporate managed device because of limited storage and restrictions on managing these environments. Running out of disk space can cause your computer to crash. You may need to remove WSL or ask IT support for help with WSL disk optimisation or compression. See WSL2 disk keeps growing? Here's how to shrink it - DEV Community.

To set up a sandbox, follow these steps:

1. Install Ubuntu on WSL (via PowerShell or Company Portal).
2. Install the Dev Containers extension in VS Code.
3. Within WSL, install Docker and test that it runs.
4. Test the end-to-end setup by cloning the "User Research Project Template" from GitHub and opening it in a dev container. This should run an isolated environment with a mounted folder that preserves Copilot chat.

AI hub

Using artificial intelligence to support design

Introduction

Designers are encouraged to experiment with AI tools, referencing this guidance and applying critical thinking throughout the process, and generously sharing experience, examples, and insights with the community.

As more designers use AI tools, we expect this guidance to evolve, expand and change over time

Create digital prototypes using AI

Opportunities for AI use in prototype creation

Opportunity	How you can use AI	Impact
Rapid creation of demos, clickable prototypes and mock-ups	Using Figma Make you can generate low and mid-fidelity mock-ups to support conversations and ideation	Reducing the time it takes to get alignment on ideas and direction
Allowing non-developers or non-coding designers to participate more directly in creating functional prototypes	Using Figma Make, VS Code with GitHub Copilot, or MS Copilot, you can generate code including using the NHS Prototyping Kit	“Vibe-coding” can reduce the time taken to create coded prototypes, and increasing the quality of our coded prototypes
Support designers with ideation	Using AI to generate ideas of how to solve problems based on best practice or established patterns from other contexts or countries based on well-crafted prompts	Faster idea generation to create a wide range of ideas during the “diverge” part of the Double Diamond.
Support with coding, especially understanding structure and limitations	Asking AI to explain how to compose a view, or to explain the different paradigmatic approaches	Reducing the learning curve for designers to create effective prototype code with the NHS Prototyping Kit
Help with understanding code structure and limitations (i.e. asking AI to explain how to compose a view, or to explain the different paradigmatic approaches, etc.)	Asking AI to explain how to compose a view, or to clarify different programming paradigms and architectural approaches. This can be done by prompting AI tools to break down the code, highlight best practices, and outline structural considerations—especially using the NHS Prototyping Kit as a reference.	Reduces the learning curve for designers and developers, helping them to create more effective and efficient prototype code. This support leads to better understanding of technical constraints and possibilities, resulting in higher quality prototypes and faster upskilling.
Speed up coding	Enabling Copilot (and others) in VS Code and allowing it to autocomplete lines of code	Write code faster, get a working prototype faster

Opportunity	How you can use AI	Impact
Troubleshooting bugs in code	Asking AI to review code and the error message it has given for faster fixing of code	Find and fix bugs faster, get a working prototype faster.
Review code prototypes against the structured logic of WCAG 2.2, to diagnose potential accessibility issues.	Asking AI to review prototype against WCAG2.2 AA standard, or the NHS Accessibility Checklist	Improve accessibility of prototypes for user research or support production coding to be more accessible.

How to prototype in code

Using AI to support design and prototyping is sometimes referred to as ‘vibe coding’ - a fast, intuitive approach where designers or developers enter high-level prompts and receive generated code or prototypes to review and iterate on.

Using enterprise licence for GitHub Copilot lets us create supportive environments, like training Copilot to build prototypes with our prototype and front-end kits and pattern libraries, and to check for WCAG 2.2 AA accessibility adherence and highlight any gaps.

It is recommended to add comments in the code to show which parts of the code were AI-generated.

For detailed steps to set up GitHub Copilot, follow the guidance in Get started with GitHub Copilot.

Prototyping in Figma

Figma Make is an AI-feature that can create a prototypes using Prompts rather than manually creating prototypes.

To use AI to prototype using Figma, you can use Figma Make. It might be useful for concept ideation.

Also if you are not confident coding a prototype using the primary toolset (NHS Prototype Kit, Github, Heroku), then Figma Make may be a suitable compromise.

However, there are some limitations to be aware of:

- Figma Make prototype won't match the NHS Design System / UI patterns
- Code output does not integrate with the NHS prototyping kit, and may not be feasible for deploying in live services
- The code can also be difficult to debug and read
- Limited accessibility support

- Data hosted outside of EU - avoid any personal data being uploaded into the tool
- If credits run out, the team may not be able to further modify their prototype

To get licence to Figma, follow guidance on the Get access to tools and software for research and design on the UCD Centre of Expertise (CoE) SharePoint.

Each Figma account has monthly Figma AI credits limit

- Full seats have 3,500 credits per month.
- Free seats have 500 credits per month

Once credits have run out no further credits can be consumed until credits reset for the next month. Unused credits do not carry over.

AI hub

Using artificial intelligence to support user research

Introduction

In August 2025, our guidance stated that no artificial intelligence (AI) tool was approved for processing participant data. This guidance was revised (November 2025) to align with more flexible approaches currently being adopted across other government departments. In February 2026, the user-research guidance has been broadened to combine with guidance for designers.

As NHS England, we are committed to enabling professional growth through experimentation, but safety and safeguarding must always come first.

We have been evaluating a variety of AI tools during the last year, and in September 2026 we have released additional guidance on Processing user research data with AI Tools.

Despite the guidance and tools being relaxed further, a key principle remains the same:

Researchers remain fully accountable for the accuracy and integrity of their analysis and reporting. Any errors or biases resulting from the misuse of generative AI are their responsibility.

A summary of key changes that apply to processing raw research data (updated September 2026):

1. approved Microsoft tools may help create anonymised derivative research data;
2. humans MUST approve it;
3. Microsoft tools or GitHub Copilot can analyse human-approved anonymised derivative research data;
4. all final outputs must be human-reviewed before publication.

Examples where using AI is NOT allowed for research purposes

1. **Do not input official or sensitive NHS England data into (non-corporate approved) AI tools.**
   - Unannounced policies.
   - Internal communications.

   Reason: This prevents disclosure of confidential information outside the organisation.

2. **Do not use AI to simulate research participants (e.g. in interviews, usability testing, surveys, card sorting, AB testing, personas, etc.)**

   AI simulations cannot replace real user insights and are likely to exacerbate health inequalities. AI-generated data are likely biased and lack grounding in research evidence. They risk reinforcing stereotypes, misinterpreting user needs and exclude insights from people who face the most barriers to accessing our services.

   - Humans bring lived experience, emotions and context that shape their behaviour. AI generates responses based on patterns in data it was trained on. It can approximate behaviour, but it doesn't experience things.
   - Humans interpret meaning based on personal, cultural and situational context. AI can simulate context if explicitly described. But otherwise, it might miss cues and overgeneralise.
   - Human participants bring diverse perspectives shaped by their identities, environments and histories. AI can simulate diversity, but it's limited by the data it was trained on and the assumptions included into its prompts.

   Reasons: Simulated responses are not evidence-based and can mislead design decisions. Human participants may disagree, challenge assumptions or describe unexpected experiences. AI models often exhibit sycophantic behaviour, producing responses that appear helpful or agreeable rather than accurately reflecting real-world experiences.

3. **Do not use AI to make ethical or safeguarding decisions that require human judgement.**

   Human oversight is essential for decisions involving participant or patient safety and wellbeing. AI cannot fully understand context, nuance, or moral implications.

   Reason: Relying on AI for these decisions risks overlooking subtle indicators of harm or bias, which could lead to unethical outcomes or safeguarding failures. Always involve a qualified person in making these judgments.

Examples where AI is allowed for research purposes

What you can and cannot do with AI depends significantly on the task. In this section, we outline practical examples for two key stages: planning user research and analysing user research.

Please read both carefully, as the rules differ. In particular, pay close attention to the guidance on analysis—this is where the highest risks lie, and not all uses are permitted. Performance will vary across different AI tools and tasks.

There is also new guidance on UR data storage and AI tools usage for user research: Processing user research data with AI Tools.

Planning research

Below are examples of planning tasks where AI can assist. This is not an exhaustive list:

- **Refining research plans**: improve problem statements, aims, objectives, and assumptions.
- **Improving discussion guides**: refine questions or prompts for interviews and workshops.
- **Enhancing research materials**: review and improve participant information sheets, consent forms, posters, and flyers.
- **Proofreading communications**: check spelling, grammar and tone in emails to participants and recruiters.
- **Planning workshops**: suggest timings, icebreakers, and clear instructions for activities.
- **Improving survey questions**: refine wording for clarity and neutrality.
- **Gathering background information**: use AI to get an overview of a topic—but proceed with caution. AI-generated content may include inaccuracies or fabricated details. If you use AI for this purpose, choose tools that provide verifiable sources and always cross-check the information.

Analysing research

Researchers are now permitted to use Microsoft 365 Copilot to assist with the analysis of user research data, including personal data.

Please ensure you are logged into Copilot using your NHS.net Connect account. This enterprise version of Copilot is not the same as the copilot you can access freely on your personal devices.

Copilot should never replace human judgment, do not use it for making decisions like which user needs are most important, or which features should we recommend. As an NHS England researcher, you remain fully accountable for the outputs of the research.

More details on how Copilot works can be found on Microsoft Learn. In summary, when you use Microsoft 365 Copilot:

- Your data stays within the Microsoft 365 boundary.
- Your data is secured based on existing security, compliance and privacy policies already deployed by your organisation.
- Prompts, responses and data accessed through Microsoft Graph aren’t used to train foundation LLMs.
- Microsoft 365 Copilot uses Azure OpenAI services for processing, not OpenAI’s publicly available services. Azure OpenAI doesn't cache customer content and Copilot modified prompts for Microsoft 365 Copilot.

Before you start using Copilot please read the M365 Copilot Acceptable Use Policy.

How to use Copilot to assist with analysis?

You can use Copilot for analysis only with anonymised data and under the following conditions:

- **Review process**
  - Researchers must review all outputs in full.
  - Researchers should never rely on AI-generated outputs without checking them thoroughly.
  - For large survey data sets it may not be feasible for a researcher to check the output of the AI tool analysis. Therefore researchers should make an informed decision in the research planning stages whether the perceived benefits are worth it.
  - Validate all findings, insights and quotes against the original data to ensure accuracy and context.
  - Never publish any outputs that have been written 100% by AI tools.
- **Disclaimer in research reports**
  If AI has been used to assist with analysis and write-up of the report, include a disclaimer at the beginning of the report stating:
  - Which AI tool was used (including version)
  - How it was used, e.g., to identify key findings and insights, search for quotes, check grammar etc. Example text: *"This report was generated with the help of [insert tool and model version here] and carefully reviewed by our UCD team for accuracy and appropriateness. AI was used to [insert brief description here]."*
- **Participant consent and transparency**
  To ensure ethical and transparent use of AI in research analysis:
  - **Include a clear statement about AI use.** Recommended wording: *"AI may be used to support the user research analysis process. None of your personal data will ever be imported into an AI tool. Only anonymised data (i.e. survey responses or transcripts) will be used with AI. All analysis using AI will be reviewed and assessed for accuracy by a member of staff."*
  - **Use updated templates.** Standard participant information sheet template will be updated shortly to reflect these additions.

  As per Market Research Society guidance a decision was made that we should be transparent about AI use so that people:
  - Are aware that AI may be used
  - Know how it may be used

- Can ask questions about how it is used

However, participants don't need to consent to AI being used to analyse their data.

You should be very cautious using AI for these parts of the research analysis process:

- **Recommendations:** Recommendations for product or service changes must be written by the researcher or UCD team members. Even if AI assisted with analysis, it may lack critical context and team knowledge. Writing recommendations is one of the most important responsibilities of a researcher—outsourcing this task would be highly risky.
- **Personas:** Unless the AI tool you are using can provide exact citations for the sources behind its insights, creating personas with it is risky because it becomes difficult to verify how they were generated and whether they embed biases. In general, views on the value of personas in user research are mixed. While some teams find them useful, others question their effectiveness.

Next read

Processing research data with AI Tools

More resources

Prompt Frameworks

- Library & Knowledge Service - MWL | AI Prompt Library
- 20 Prompt Engineering Frameworks: Complete Guide (2026)

AI for User Research: Current State and Key Limitations (Feb 2026)

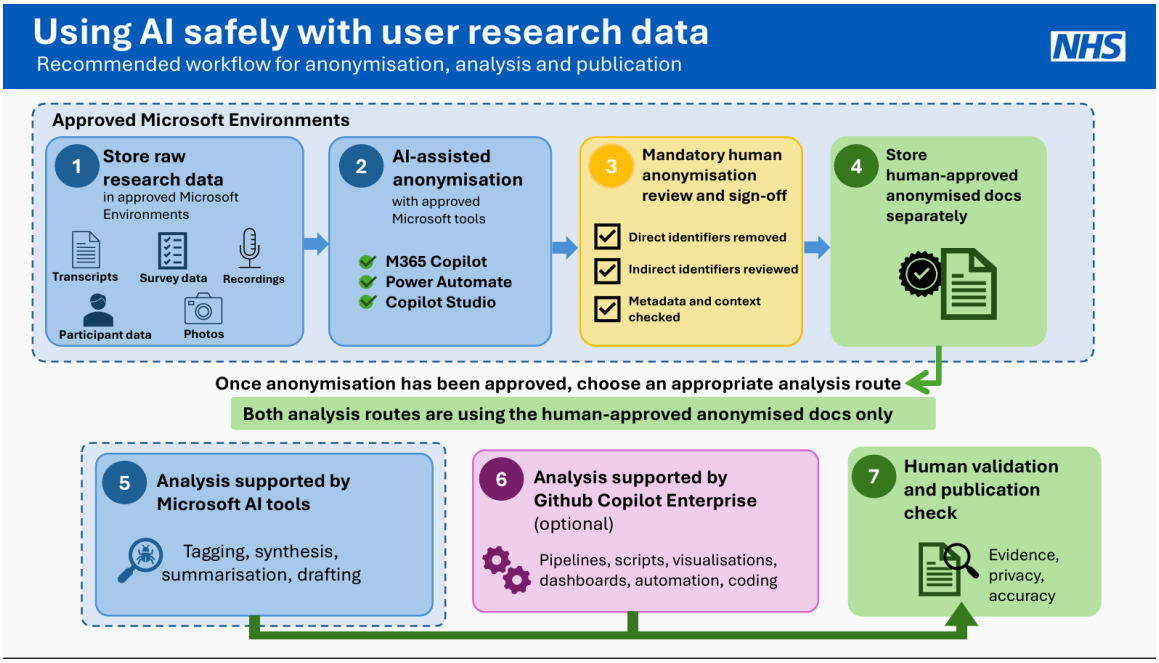
Processing research data with AI Tools

This page expands on Using artificial intelligence to support user research to support colleagues to work safely and ethically when processing research data.

Key rules that apply to processing research data:

1. approved Microsoft tools may help create anonymised research data
2. a human must review and approve the anonymisation
3. Microsoft tools or GitHub Copilot can analyse human-approved anonymised research data
4. all final outputs must be human reviewed before publication

Revised end-to-end research workflow



Approved Microsoft tools may process raw research data to support anonymisation. A human must review and approve the anonymisation before the data is used for analysis. Both Microsoft AI analysis and GitHub Copilot Enterprise analysis must use human-approved anonymised data only.

1. Store raw research data in an approved Microsoft environment

Raw transcripts, participant lists, survey exports, recordings, photographs and other identifiable research materials should remain in the approved NHS England storage environment, with access limited to authorised colleagues.

Approved environments include: SharePoint (restricted sites/folders), restricted Teams sites, Power Platform (e.g. Dataverse).

Using personal OneDrive to store research data is generally not recommended due to business continuity risks.

NHS England policy requires people to use approved ICT appropriate to the classification of the information and to manage records in accordance with records-management policy. See Transparency and assurance.

2. AI-assisted anonymisation with approved Microsoft tools

Microsoft AI tools can be used to anonymise raw research data. An approved workflow may use:

- Microsoft 365 Copilot within NHS.net applications
- Power Automate, including an approved AI Builder or document-processing workflow
- Copilot Studio, where the use case has an approved NHS England environment, licence, connectors, data-loss-prevention configuration and governance

See Artificial intelligence tools for the list of approved tools.

Anonymise transcripts

When anonymising transcripts, apply the appropriate level of anonymisation based on the risk of disclosure and the need to preserve data utility.

Avoid both over-anonymisation (removing too much detail, making data less useful) and under-anonymisation (leaving identifiable details that risk disclosure). Remove indirect identifiers: job titles, unique events or other details when they might collectively reveal an identity.

Best practice for text-based data:

- Make sure that you apply consistent anonymisation across all transcripts
- Clearly mark replacements (e.g., with [square brackets])
- Remember to keep a secure record of pseudonyms if needed for analysis. Any document that contains participant or patient personal information should be stored in a secure location

Examples of anonymisation techniques (not exhaustive):

- Replace a person's name with a pseudonym, e.g., [Participant 1] or a descriptive tag, e.g., [farmer]
- Replace geographical locations with a generic tag, e.g., [town in East of England] or [participant's hometown]

- Replace addresses, workplaces or dates of birth with tags, e.g., [address removed]
- Remove or generalise unique identifiers such as job titles, rare conditions or specific events that could reveal identity

The UK Data Services learning hub has some very comprehensive guidance on how to anonymise qualitative and quantitative data.

See the UCD Prompt Library for examples of how to anonymise data.

3. Mandatory human anonymisation review and sign-off

AI-assisted anonymisation must not be treated as complete automatically.

A human researcher **MUST** review and approve the anonymised output before it is used for analysis, synthesis, reporting, sharing, publication or transfer. The reviewer should check direct identifiers, indirect identifiers, metadata and combinations of information that could enable re-identification.

The Microsoft output is an anonymisation "candidate for review", not automatically a safe anonymised dataset.

An accountable human researcher must verify that:

- direct identifiers have been removed or replaced
- indirect identifiers and combinations of details have been assessed
- pseudonym keys are not included
- filenames, comments, document properties and embedded content do not reveal identity
- useful research context has not been incorrectly removed
- the approved anonymised document is clearly distinguished from the raw source
- the review and approval are recorded

This control reflects the AI guidance requirement that users validate AI output before using it and remain responsible for the final work product.

4. Store approved anonymised documents separately

Only after human sign-off should the anonymised document be labelled or treated as suitable for:

- AI-supported analysis and synthesis
- transfer to an approved GitHub repository
- GitHub Copilot-supported coding or analysis

- report drafting
- wider internal sharing, subject to the intended audience and permissions

The raw source remains subject to its existing access, retention and disposal arrangements. Prompts, responses and AI-created documents may themselves be corporate records and may be searchable or disclosable under subject-access or FOI processes.

Raw participant material; e.g. raw transcripts, recordings, participant lists, identifiable survey data, photographs containing personal data, pseudonym keys or other identifiable or confidential research material, **must not be used in the AI-analysis stage**.

Recommended environments and folder storage pattern

Approved Microsoft 365 research storage

1. Team restricted User Research area
   - `/Raw` – restricted original research data
   - `/Anon_Pending` – AI-assisted anonymisation candidates ready to be checked
2. Wider team storage / UCD SharePoint
   - `/Anon_Approved` – human-reviewed and signed-off anonymised research data
   - `/Analysis`
   - `/Reporting`
   - `/Project docs`

Approved GitHub Enterprise (optional)

3. Project GitHub repository
   - `/Anon_Approved` – human-reviewed and signed-off anonymised research dataset
   - `/Analysis scripts`
   - `/Tests`
   - `/Outputs`
   - `/Docs`
   - `.gitignore` – this file can exclude `/Anon_Approved` and `/Outputs` folders being uploaded to GitHub

Do not create `/Raw` or `/Anon_Pending` data folders inside a GitHub repository or within a workspace routinely exposed to GitHub Copilot.

5. Analysis: supported by Microsoft AI tools

Microsoft Copilot (Premium) is integrated within all Office 365 applications. There are a variety of use cases that are available to support researchers, and we encourage teams to share their process on the UCD Prompt Library.

Some examples of useful and safe tasks M365 tools can support with access to human-approved anonymised research data:

- Research planning, including supporting ethics and safeguarding reviews
- Finding quotes and tagging metadata
- Aggregating data across several interviews
- Tagging and analysing quantitative data / survey responses
- Writing analysis scripts
- Supporting analysis in M365 products (e.g. Excel, Power BI, PowerPoint)
- Summarising interviews / drafting text for reports
- Creating personas / visualisations / workflow and process maps / infographics
- Editing video and audio recordings, creating highlight reels (Clipchamp)

Examples of Microsoft 365 Copilot tasks

Raw research data can be used where authorised and appropriate within the approved Microsoft environment. Analysis should use human-approved anonymised data.

- Review this raw interview transcript and create an anonymised version. Remove or replace direct and indirect identifiers while preserving information that is important to the research. Highlight anything you are uncertain about so I can review it before approving the anonymised version.
- Review these human-approved anonymised interview transcripts. Identify recurring themes, differences between participants and supporting quotes. For every finding, show me which source transcript it came from so I can verify it.
- Analyse this human-approved anonymised Excel survey dataset. Summarise the main patterns and differences between responses, identify anything unusual that I should investigate, and show the evidence supporting each observation.
- Using these validated research findings, draft an executive summary for my research report. Clearly distinguish evidence from interpretation and do not introduce findings that are not present in the source material.
- Turn these validated research findings into a PowerPoint presentation for stakeholders. Summarise the key user needs, evidence, implications and recommendations, and keep links to the supporting research where possible.

- Compare the findings in these human-approved anonymised research reports and identify themes that appear across multiple studies. Show which reports support each theme and flag any contradictory evidence.
- Review this draft research report before publication. Identify any information that could directly or indirectly identify a participant, any unsupported claims or quotations that need checking, and any content that may need a copyright or source check.
- Summarise the validated findings from this research into a short update for the project team, including the strongest evidence, important limitations and the decisions that still require human judgement.

6. Analysis: GitHub Copilot Enterprise (optional)

GitHub Copilot may only be used through NHS England's Enterprise GitHub arrangement and only with data that has already been anonymised and approved by a human reviewer. See Get started with GitHub Copilot.

GitHub Copilot is primarily a coding agent (used in Visual Studio Code), however, it is also useful for long-running complex analysis, multi-step tasks and data summarisation.

It is similar to MS Copilot Researcher or Analyst Mode, however there is a wider choice of Large Language Models, and code is generated that can be reused, tested and validated. GitHub Copilot provides a flexible model where you can use different (OpenAI, Anthropic, or Microsoft) LLMs, it is a collaborative experience where the agent will request clarifications and agree next steps, and you can see the instructions explaining how the AI agent is processing your requests. By comparison, M365 Researcher/Copilot/Chat often complete data processing behind the scenes, and provide a finished artefact without explanations or justification.

To get started, clone: Template GitHub repository with example test data

Examples of GitHub Copilot Enterprise tasks

Use human-approved anonymised research data only.

- Review these 10 anonymised transcripts. For each transcript, identify themes and supporting quotes, then save the results into a structured CSV file so I can review them.
- Create a reusable Python script to analyse my monthly anonymised survey feedback. Include data-quality checks, theme counts and a summary table, and document how the analysis works.
- Help me design and build an API connector for Qualtrics that downloads my project data each month.
- Build a locally hosted dashboard for this anonymised dataset using NHS design and accessibility standards. Include filters, charts and an option to export the

underlying results.

- Review this directory containing 30 research reports. Extract the user needs from each report using this definition of a user need: […]. Harmonise overlapping needs into a single set, retain links to the source evidence, and create a table that I can review and prioritise.
- Review my Power Automate solution ZIP file, identify why the cloud flow is failing, suggest a fix, update the solution and show me how to test it before I re-import it.
- Create a reproducible analysis pipeline that takes an approved anonymised CSV, cleans and validates the data, runs the agreed analysis and generates the same tables and charts each time.
- Create an interactive visualisation that helps us explore relationships between these anonymised user needs, research themes and service areas. Keep the underlying analysis transparent so researchers can verify how each result was produced.

When using GitHub it is recommended to

- Create projects in Organisation folders (NHS Digital, NHS England Tools or NHS.UK) when you are collaborating with colleagues
- Ensure your GitHub Business/Enterprise account is logged in, so data is protected in line with the GitHub Copilot contract

We have zero-data-retention arrangements for many generally available models, as well as restrictions preventing LLM providers training new models on NHS data (however there are some preview-specific exceptions).

Appropriate user researcher uses include

- writing or improving analysis scripts
- developing repeatable analysis pipelines
- testing transformations and data-quality checks
- creating visualisations, prototypes or innovative outputs
- automating repeatable processing
- supporting report structures or drafting text
- documenting and explaining code

GitHub Copilot must not

- be used to anonymise data – we can’t share any personal identifiable data or Sensitive or Official Sensitive data in GitHub Copilot Enterprise
- be treated as an autonomous researcher – replacing researcher judgement, evidence validation or interpretation

- be used as a research repository / insight store library
- be careless about which files you push into GitHub. Only push reusable code, docs and agents to GitHub

Avoid pushing project data (research data and findings) into GitHub storage. A minimum approved anonymised dataset may be used for a task, but GitHub must not become the authoritative research repository. Research insights and artefacts **MUST** be stored and shared in SharePoint/Teams/Confluence folders etc.

GitHub Copilot limitations

- GitHub Copilot doesn't have access to Microsoft Graph (SharePoint, OneDrive and 365 apps) by default – data access needs approval by the user, on a per repository, file or folder basis
- Data will be sent outside the UK (the US specifically, if using frontier OpenAI / Claude models)

It is important to test different versions of prompts and different LLM models, as they will return different outputs. Again, we encourage teams to share their workflows on the UCD Prompt Library.

Controls

Below are controls teams can put in place to support security of research data and compliance with Information Governance requirements.

Control	Value	Position
RAW and ANON_PENDING in M365	Makes workflow state clear while retaining raw data within the approved Microsoft boundary.	Recommended
ANON_APPROVED in M365	Creates a controlled hand-off point after human sign-off.	Recommended
Separate clean GitHub workspace	Minimises accidental attachment or agent access to unredacted raw material data by keeping working files separate from raw documents. A minimum approved anonymised dataset may be used for a task, but GitHub must not become the authoritative research repository.	Recommended
Use an isolated virtual machine	Running GitHub Copilot in a virtual machine (dev containers) can reduce risks around files being accessed outside the repository workspace.	Recommended
.gitignore for local temporary files	Useful repository hygiene. e.g. add .gitignore exclusions for /ANON-APPROVED and /Outputs folders to prevent temporary research data being uploaded to GitHub.	Secondary control only
Copilot instruction “use only approved anonymised data”	Reinforces expected behaviour.	Useful but not a security boundary
GitHub content exclusion	Repository owners can prevent selected files from being used by some GitHub Copilot features. This provides an additional safeguard, but it should not be used to protect raw or identifiable research data.	Defence in depth only
RAW data inside GitHub repo plus .gitignore	Reduces accidental commits but does not prevent all development environments, agent or user access.	Not recommended

To get started, clone: [Template GitHub repository with example test data]

7. Human validation and publication check

A researcher must validate AI-generated analysis, summaries, themes, quotations, calculations and interpretations against the original research evidence. Researchers remain accountable for themes, findings, recommendations and final outputs.

The NHS England guidance says trends and insights identified by AI tools must be verified and validated, and that AI-generated documents are a starting point rather than a finished product.

Before publication or wider distribution, conduct a separate check for:

- personal or confidential information
- re-identification through quotations or combinations of details
- accuracy and evidential support – i.e. check for hallucinations
- appropriate source attribution – check references
- copyright and other people's content: before publishing AI-assisted work, check that you have permission to use any externally owned text, images, code, datasets or other material. Attribute sources where required and seek advice if you are unsure
- accessibility and intended audience
- appropriate disclosure of AI assistance

Examples of AI disclosure statements

- **AI-assisted research:** "Approved NHS England AI tools supported parts of the anonymisation, analysis and drafting process. Human researchers reviewed the anonymisation, validated findings against the research evidence and approved the final report."
- **Use of AI:** "Microsoft 365 Copilot was used to support anonymisation and analysis of this research. GitHub Copilot Enterprise was used with human-approved anonymised data to support analysis scripts and visualisations. Researchers reviewed and validated all AI-assisted analysis, findings and outputs before publication."

Risks

Using AI throughout the whole research lifecycle may have benefits, but it also introduces a number of considerable risks. It is important for researchers and colleagues to be aware of these risks, apply necessary mitigations to ensure research data is protected and research quality standards are maintained.

Risk ID	Risk area	Risk statement	Controls / mitigations	Owner
UR01	Unapproved anonymisation tool	Raw participant data is submitted to a public, personal or otherwise unapproved tool, causing unauthorised disclosure.	Use only NHS England-approved Microsoft accounts, environments and workflows. Never use public AI for raw research data.	Researcher
UR02	Incomplete anonymisation	Microsoft 365 Copilot, Power Automate or Copilot Studio misses direct or indirect identifiers, causing identifiable data to be treated as anonymous.	Treat output as a candidate ready for review only. Require documented human review and sign-off before analysis, sharing or GitHub use.	Human reviewer
UR03	Over-anonymisation	AI removes context needed to understand accessibility needs, health experiences or service barriers, weakening research validity.	Compare the anonymised data with the source; restore necessary non-identifying context; preserve a protected source copy.	Researcher
UR04	Pseudonym-key disclosure	The anonymised dataset and re-identification key are stored together or moved into GitHub.	Store the key separately in approved Microsoft storage with restricted access. Never place it in GitHub.	Researcher
UR05	Raw data enters GitHub	A transcript, recording, participant list, identifiable survey export or photograph is committed or exposed to GitHub Copilot.	Raw data remains outside GitHub and GitHub workspaces. Permit only human-approved anonymised data. Conduct a pre-commit review.	Researcher

Risk ID	Risk area	Risk statement	Controls / mitigations	Owner
UR06	Metadata leakage	Names or confidential details remain in filenames, properties, comments, commit messages, branches, issues or repository history.	Include metadata in the anonymisation sign-off; use neutral filenames; review commits, issues and pull requests before pushing.	Researcher / repository owner
UR07	Inadequate repository access	A repository is public, placed in the wrong organisation or accessible to unnecessary users.	Use an approved NHS England GitHub organisation and enterprise account; apply private/internal visibility and least-privilege access; review collaborators.	Repository owner
UR08	Overseas GitHub processing	An anonymised document, prompt or output is processed through non-UK model infrastructure.	Use pre-anonymised data only; apply enterprise-approved models and features; do not represent Zero Data Retention as equivalent to UK processing.	GitHub administrators / AI user
UR09	Control overconfidence	Users assume .gitignore, Copilot instructions or content exclusion make raw data safe.	Treat these as defence-in-depth only. Physical separation and pre-anonymisation are the primary controls. GitHub notes that content exclusion does not cover every agent mode and may not prevent indirect semantic context reaching Copilot.	Repository owner

Risk ID	Risk area	Risk statement	Controls / mitigations	Owner
UR10	Analysis error or bias	AI invents, omits or overstates themes, quotations, sentiment or findings.	Validate material outputs against source evidence; require researcher interpretation and peer challenge; do not automate decisions about individuals.	Researcher
UR11	Unsafe generated code	GitHub Copilot creates faulty transformations, insecure code or an analysis pipeline that silently alters data.	Review, test and peer-review generated code; use known test cases or synthetic data; preserve reproducible steps and version history.	AI user / technical reviewer
UR12	Prompt and output handling	Prompts, anonymisation candidates or generated outputs are retained or shared without appropriate controls.	Treat them as records; store final approved outputs in the appropriate NHS repository; follow access, retention and disposal requirements.	AI user
UR13	Publication disclosure	A report, presentation, dashboard or repository contains personal data, identifiable quotations or confidential information.	Mandatory pre-publication privacy, evidence and audience check by a named human reviewer.	Report owner / team lead
UR14	Intellectual property	AI-generated text, visualisation or code reproduces protected content or introduces incompatible licensing.	Check sources and licences; attribute appropriately; escalate uncertainty. NHS guidance requires users to ensure AI output does not breach third-party IP rights.	Report owner / team lead

Risk ID	Risk area	Risk statement	Controls / mitigations	Owner
UR15	Over-reliance	Researchers delegate interpretation or recommendations to AI and lose familiarity with participant evidence.	AI supports, rather than replaces, researcher-led synthesis, judgement, recommendations and accountability.	Researcher / research lead

AI hub

Using artificial intelligence to support content design

Placeholder page - Coming soon!

Standards to follow

UK government guidance

- **Updated Standards AI Guidance (Feb 2026)**

  As we see more and more Services that utilise AI the service standard has been updated to be clearer about requirements around the use of generative artificial intelligence (AI) within services:

  - Point 2, 'Solve a whole problem for users', is clearer that not designing services around technologies includes not designing them around AI technologies. And that checking for existing solutions includes existing AI solutions
  - Point 6, 'Have a multidisciplinary team', now requires service teams using AI in a service to have someone on or available to the team who fully understands how it works and the impact of using it.
  - Point 11, 'Choose the right tools and technology', makes it clearer that understanding technologies includes understanding AI technologies. Service teams are also now required to assess the impact of technologies on user experience, inclusion, and the reliability of information and decisions.
  - Point 14, Operate a reliable service', now requires quality assurance testing to not be left to automated tools. Service teams must also now monitor outcomes for users and ethical issues such as bias.

    Please look through the Service Standard for more details and please reach out to DHSC Technology Assurance team if you have any questions.

- **Artificial Intelligence Playbook for the UK Government (2025)**. Offers a comprehensive framework for using AI across government departments. Includes principles, ethics, governance, risk management, and case studies.
- **Guidance to civil servants on use of generative AI (2024).** Provides principles and expectations for how UK civil servants should use generative AI tools responsibly.
- **AI in government knowledge hub**. Provides guidance, practical tools, and real-world case studies to help public sector teams adopt and use AI responsibly. It includes a Use Case Library and a Prompt Library https://ai.gov.uk/knowledge-hub/prompts/.
- GOV.UK Technology Code of Practice (**The Technology Code of Practice - GOV.UK**).
- **The Public Sector Bodies (Websites and Mobile Applications) (No. 2) Accessibility Regulations 2018**

NHS guidance

- **Using AI in NHS England** (January 2026)
- **NHSE AI Acceptable usage policy** (December 2025)
- **NHS Design Principles**
- **NHS Service Standard**
- **Engineering AI Guidance** & **Tech Radar**
- **Artificial Intelligence (AI) and Machine Learning Guidance (2023–2025)**. Guidance for developing and adopting AI, especially in general practice. Covers use cases, information governance, clinical validation, and workforce readiness.
- **Artificial intelligence (2025).** Focused on using AI in NHSE's Transformation Directorate. It provides information governance (IG) guidance for using AI lawfully and safely in NHS services. It addresses roles and responsibilities for patients, healthcare workers, and IG professionals, including legal bases, consent, data minimisation, and transparency.
- **Regulation of AI (including medical devices) in Healthcare - GOV.UK**
- **Using AI safely and responsibly in primary care - The MDU.** This article from the Medical Defence Union (MDU) outlines the safe and responsible use of AI in primary care, emphasizing the growing role of AI in diagnostics, decision support, and administrative tasks. It warns that using AI outside approved governance frameworks can pose significant legal, ethical and professional risks, including data breaches, misinformation and bias.

Transparency and assurance

Key points:

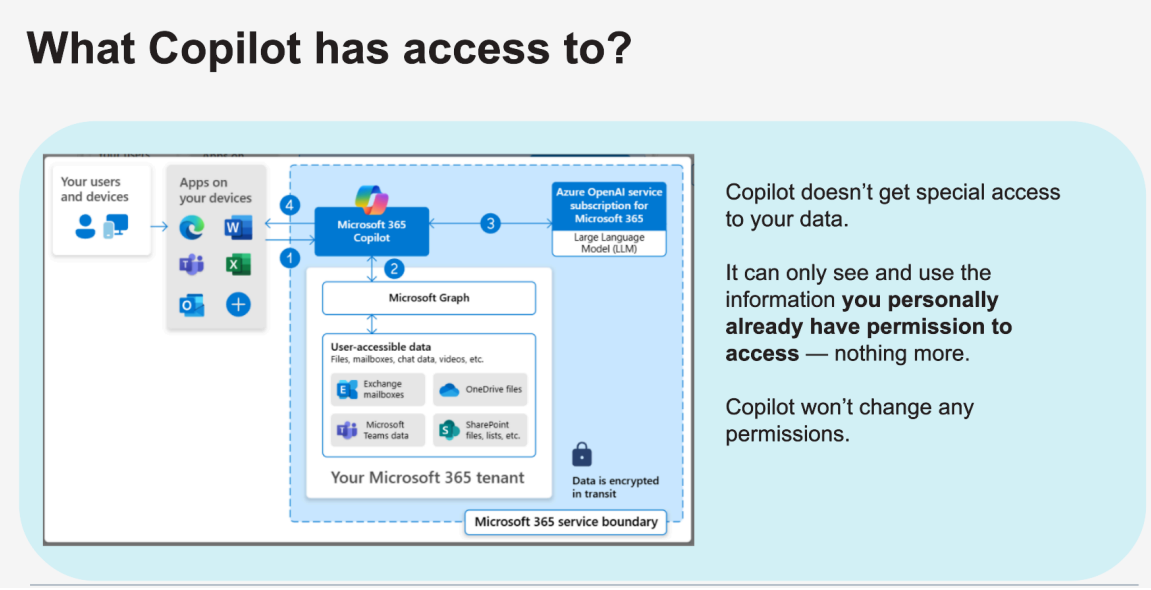
- You can generally use Approved AI Tools in line with the guidance.
- For use-cases that might be considered higher risk, or that might contradict guidance, you should ask for technical support or a second opinion.

Assurance processes that ALWAYS need to be followed

- Technical reviews and spend approvals if you want to spend any money on an AI Tool (or service) with AI integrated.
- Standard approvals processes (Technical Review Group Home, GAtS (Get Approval to Spend) service) are needed.
- Commercial and financial approval.
- DHSC service technology assurance processes apply for all health services, including those integrated with AI/LLMs. The Gov.uk Service Standard was updated recently to include use of generative AI within services. If you are unsure if your services needs DHSC review, contact DHSC Technology Assurance Team.

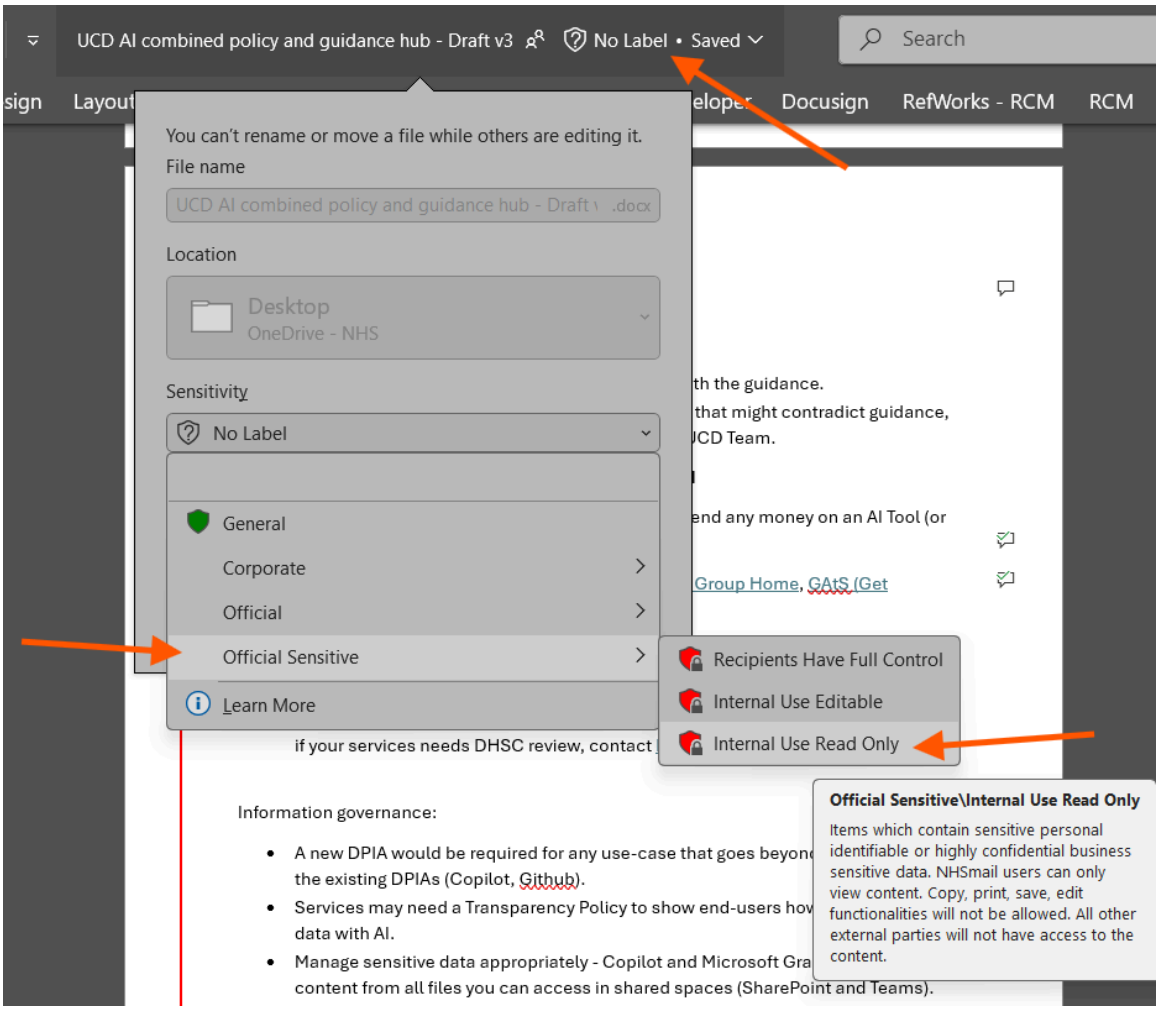
Information governance

- A new Data Privacy Impact Assessment (DPIA) would be required for any use-case that goes beyond the scope of the existing DPIAs (Copilot, GitHub, etc).
- Services may need a Transparency Policy to show end-users how they process data with AI.
- Manage sensitive data appropriately - Copilot and Microsoft Graph can return content from all files you can access in shared spaces (SharePoint and Teams). Retention labels alone do not prevent Copilot from accessing or returning a file in search results.



Overview of which data Copilot M365 can access.

- It is recommended to add Sensitivity Labels of "Official Sensitive – Internal Use Read Only" to confidential files to prevent Copilot and Microsoft graph from returning this content in search queries.



Adding Sensitivity Labels can stop Copilot from surfacing private data

Ask for technical support or second opinion

There are many places to get technical feedback or a second opinion about your usage of AI at NHS England, this includes:

- Join relevant AI Slack channels
- Contact NHS England AI Team
- Contact UCD Ops Team
- NHS England IT Service Desk – for tools specific guidance
- NHS England CISO / DPO
- Microsoft support team (Copilot and Copilot Studio)

Learning and development resources

Upcoming training events

More training events:

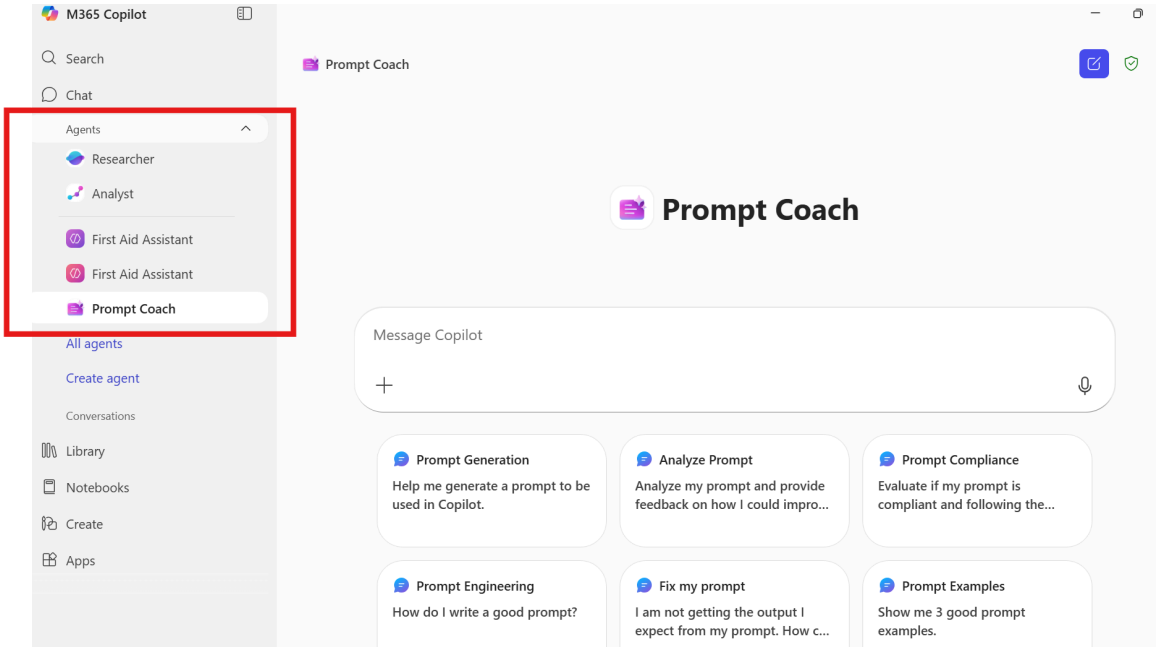
- Upcoming Copilot training sessions
- AI Public Sector Week 2026
- Increase your confidence and skills with One Big Thing's AI for All

Recommended courses

- Course Catalogue - AI Skills Hub
- Thinking Critically About AI in Healthcare - Lecture 1 - YouTube
- Learn how to use Microsoft Copilot | Microsoft Learn
- Introduction to generative AI and agents - Training | Microsoft Learn
- Power Platform in a Day Workshops | Microsoft Power Platform
- Use AI Builder in Power Automate - Power Automate | Microsoft Learn
- Microsoft Virtual Training Days | Microsoft Events

Recommended reading

- NHS AI Dictionary. An online glossary that explains key terms and concepts related to AI in healthcare. It's designed to help NHS staff and partners understand AI terminology in a clear, accessible way.
- NHS AI knowledge repository. Managed by NHS England AI Team. It includes helpful resources and relevant case studies.
- Evaluating gender bias in large language models in long-term care | BMC Medical Informatics and Decision Making. This research shows evidence that male summaries of long-term care records emphasized physical and mental health more and used more direct language, whereas female summaries often downplayed needs. These disparities could affect care allocation, underscoring the importance of evaluating bias in LLMs. The paper also offers a reproducible framework for assessing such bias in healthcare applications.
- Machine learning principles. Provide guidance for developers, engineers and decision-makers on how to design, develop, deploy and operate machine learning systems securely.
- The Five-Step Framework for Crafting Great Prompts. You can also chat to Copilot Prompt Coach to get help with writing effective prompts.



You can also chat to Copilot Prompt Coach to get help with writing prompts

- [X-Gov AI Digital Professions Conference (Jan 2026)]
- AI Use in User Research: Challenges and Recommendations - Ash Montes
- How I'm using Generative AI in my content design workflow - Phoebe Nightingale

Get involved sharing good practice

Sharing learnings from your use of AI

This is a very rapidly changing field. We therefore can only learn from each other if we share our positive and negative experiences of using AI for user research.

Join AI-related Slack channels

NHS England Slack - relevant channels:

- #ai-updates-and-data-news
- #ai-for-design-practice
- #ai_learning
- #ai-content-ops
- #dpsp-ai-interest-group
- #ethics_support

X-Gov Slack - relevant channels:

- #AI
- #ai-ucd
- #ai-for-designers
- #ai-and-sustainability
- #ucd-and-ai-meetups
- #PowerPlatform

Present to the community

You can present at wider meetings such as NHSE UCD community meet ups.

AI working group for URs

- We meet every two weeks – Thursday 4pm. Contact UCD Ops team or Vikki Wooding to be added to the invite. Review changes to AI policy, as well manage the UCD AI backlog.

AI prompt library

To support transparency and discussion, anyone can add prompts or comment to the UCD AI Prompt library:

- **User-centred design Centre of Expertise - UCD Prompt Library - All Items**

UCD Ops AI backlog

Links to ongoing work around AI

- Kanban: All UCD Issues and Epics - Agile Board - CDT JIRA
- Backlog

Ongoing AI tool experiments

There are several people across NHSE who are currently running experiments with AI tools for user research and design purposes.

Contact UCD Ops team (by emailing ucd@nhs.net) if you:

- Have conducted any user research-related experiments with AI tools which could be useful to include in this guidance.
- Are planning a user research-related experiment with AI tools and need advice on how to do it safely and in-line with this guidance.

Case Studies

Here are a few case studies that have been written.

- **User Research Finder -** https://www.gov.uk/government/publications/ai-playbook-for-the-uk-government/artificial-intelligence-playbook-for-the-uk-government-html#nhsuk-reviews-an-automated-reviews-moderator
- **NHS.uk website automated review moderation -** https://www.gov.uk/government/publications/ai-playbook-for-the-uk-government/artificial-intelligence-playbook-for-the-uk-government-html#nhsuk-reviews-an-automated-reviews-moderator

If you are interested in producing a case-study or blog for internal or external usage contact the UCD Ops team (by emailing ucd@nhs.net).

Also check the NHS AI knowledge repository. Managed by NHS England AI Team. It includes helpful resources and relevant case studies.